

— THAILAND · INSURANCE · 2027

Thailand insurance at the **super-aged** threshold

A demographic transition is rewriting the demand curve for life and health cover. Health riders, annuities, and the protection gap define the 2027 opportunity.

COUNTRY

Thailand

SECTOR

Insurance

PUBLISHED

May 2026

HORIZON

2027–2032

How we built this analysis

This report triangulates regulatory disclosures, industry-association statistics, demographic projections, and operator filings into a forward read on Thailand's insurance market. Every quantitative claim carries an inline source tag; **[Kira estimates]** marks figures derived from our own synthesis.

PRIMARY INPUTS

→ Regulatory & association data

Office of Insurance Commission directives, Thai Life Assurance Association and general-insurance association premium statistics, and listed-insurer disclosures.

→ Demographic & health-system reads

Population-ageing projections, universal-coverage utilisation studies, and national health-expenditure series anchoring the demand thesis.

→ Analyst triangulation

Where published figures diverge, our team reconciles to the most defensible mid-point and tags it as a KIRA estimate.

SCOPE & LIMITS

→ Unit of analysis

The Thai domestic insurance market — life and non-life — viewed through the demographic-transition lens. Reinsurance and offshore captives sit outside scope.

→ Currency & conversion

Thai baht (THB) reported as filed; USD conversions use a working rate of THB 34.5 per USD and are directional only.

→ Forecast convention

Forward figures are directional scenarios anchored to documented base years, not point predictions. Ranges signal genuine uncertainty.

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Thailand's insurance market — a demographic re-rating

A **USD 31 bn premium pool** meets a population crossing into super-aged territory. Headline growth is modest, but the mix is shifting decisively toward health riders, annuities, and retirement-linked cover. **The next 24 months decide** which carriers own the ageing customer.

INDUSTRY PREMIUM 2024

941 THB bn

+2.4% YoY [TLAA-OIC 2024]

TLAA-OIC

HEALTH & CI PREMIUM 2024

125 THB bn

+13.7%, 19.1% of total [TLAA 2024]

TLAA

POPULATION AGED 60+ 2024

20 %

13.6 m people [GISTDA 2025]

GISTDA

OUT-OF-POCKET HEALTH SHARE

11.3 %

structural private-cover gap

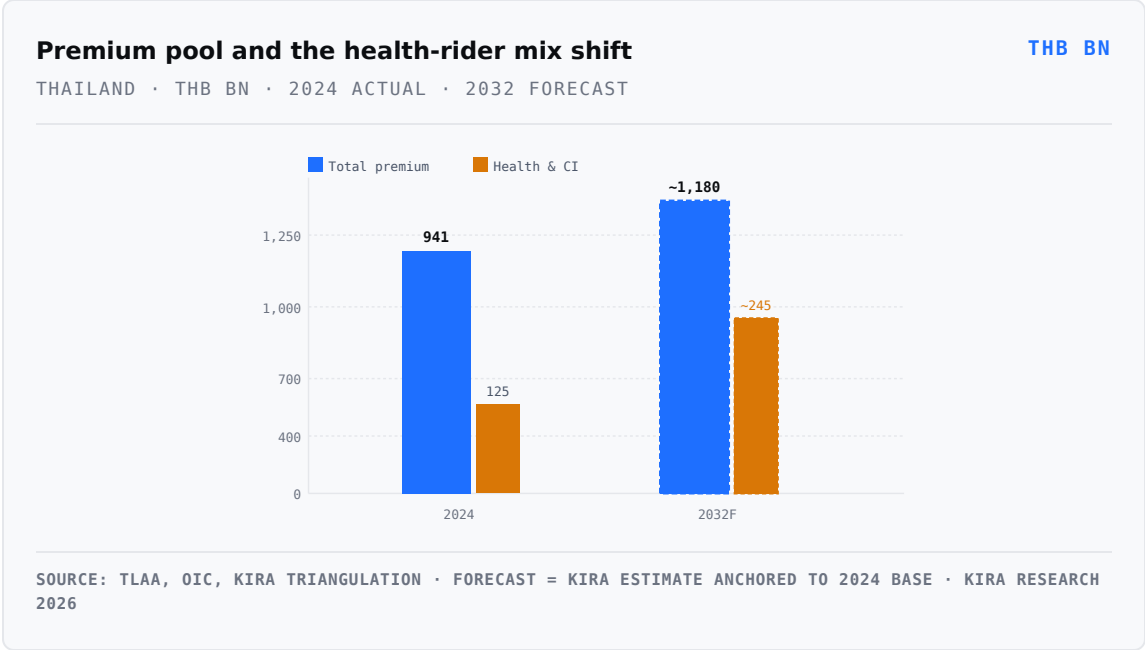
UHC STUDY

The market state

Thai insurers wrote **THB 941 bn in premiums in 2024**, up 2.4% [TLAA-OIC 2024], with life at THB 654 bn (+3.2%) [TLAA 2024] and non-life at THB 286 bn (+0.5%) [TGIA 2024]. Headline growth is single-digit, but composition is moving fast: health and critical-illness premiums jumped **13.7% to THB 125 bn**, now 19.1% of life premium [TLAA 2024].

Why now: demographics as demand engine

Thais aged 60+ reached **13.6 m, or 20% of the population, in 2024** [GISTDA 2025], outnumbering children for the first time. The country is on track to be super-aged — 28% over 60 — by 2035 [WHO 2024]. With out-of-pocket spending at 11.3% of health outlays [UHC Study 2024] and medical inflation running 8-10% [Bangkok Post 2024], the private protection gap widens each year the public scheme stays flat.



SOURCE KEY · GISTDA 2025 = Geo-Informatics & Space Technology Development Agency population analysis 2025 · TGIA 2024 = Thai General Insurance Association direct-premium statistics 2024 · TLAA 2024 = Thai Life Assurance Association annual statistics 2024 · TLAA-OIC 2024 = TLAA & Office of Insurance Commission combined industry total 2024 · UHC Study 2024 = Universal Coverage Scheme out-of-pocket study 2024 · WHO 2024 = WHO South-East Asia healthy-ageing feature 2024 · Bangkok Post 2024 = Bangkok Post insurer medical-inflation report 2024 · Kira estimates = KIRA in-house analyst triangulation

Five strategic implications for market participants

Five forces reshape competitive dynamics through 2032. Each is anchored to documented market data and our synthesis of Thailand's demographic and regulatory trajectory.

01 · DEMAND Own the ageing customer's full lifecycle With 60+ at 20% of population and rising to 28% by 2035 [GISTDA 2025] [WHO 2024], the winning carriers bundle health, annuity, and long-term care across one relationship rather than selling single products. Cross-sell depth, not new-logo count, drives 2027 value. Anchor: 13.6 m Thais aged 60+ in 2024	02 · HEALTH GAP The protection gap is the growth pool Public cover reaches 74% of people [Frontiers 2024] but leaves out-of-pocket at 11.3% of spending [UHC Study 2024]. Health and CI premiums already grow 13.7% a year [TLAA 2024]. Closing the gap for the middle class is the single largest organic opportunity. Anchor: 19.1% health share of life premium	03 · COST CONTROL Copayment resets health economics The OIC copayment clause, effective 19 March 2025 [OIC 2025], shares cost with policyholders to curb medical inflation of 8–10% [Bangkok Post 2024]. Carriers that price and communicate copay well protect loss ratios; laggards face anti-selection. Anchor: 8–10% annual medical inflation	04 · CAPITAL IFRS 17 and RBC favour scale balance sheets Risk-based capital and IFRS 17 raise the cost of long-duration guarantees [OIC 2025]. Annuity-heavy books need disciplined ALM. Sub-scale insurers face a structural capital disadvantage versus the top three; expect selective consolidation. Anchor: top 3 own ~53% of life premium	05 · DIGITAL AI moves from pilot to underwriting core AI underwriting, claims automation, and fraud models — which cut fraudulent claims ~30% in leading deployments [InsurTech 2025] — are table stakes by 2027. Bancassurance plus agent forces remain the trust channel for older buyers; digital augments, not replaces. Anchor: ~30% fraud-claim reduction
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SOURCE KEY · Frontiers 2024 = Frontiers in Public Health, low use of Thailand's Universal Coverage Scheme 2024 · GISTDA 2025 = GISTDA population analysis 2025 · InsurTech 2025 = InsurTech Digital AI claims & underwriting review 2025 · OIC 2025 = Office of Insurance Commission 2025 directives (copayment, RBC, IFRS 17) · TLAA 2024 = Thai Life Assurance Association statistics 2024 · UHC Study 2024 = Universal Coverage Scheme out-of-pocket study 2024 · WHO 2024 = WHO South-East Asia healthy-ageing feature 2024 · Bangkok Post 2024 = Bangkok Post medical-inflation report 2024

SECTION 06

The demographic transition

Thailand is ageing faster than any economy at its income level. The 60+ cohort crossed **20% of the population in 2024** and heads for 28% by 2035 — a super-aged society. This is not a backdrop to the insurance story. It **is** the story.

6.1 AGE STRUCTURE

6.2 THE SUPER-AGED PATH

6.3 REGIONAL DIVERGENCE

6.4 DEMAND CONSEQUENCES

From aged to super-aged: the demand engine for cover

The share of Thais aged 60+ doubled from 10% in 2000 to **20% in 2020-24** [World Bank 2023] [GISTDA 2025], and is projected to reach 36% by 2050. Each percentage point shifts demand from accumulation products toward health, annuity, and long-term care.

An inverted age pyramid

In 2024 the 60+ population reached **13.6 m and overtook the 9.5 m children under 15** [GISTDA 2025]. A shrinking working-age base supports a growing dependent-elderly cohort — the classic ageing-society squeeze, arriving earlier in Thailand than in regional peers.

The super-aged threshold

The WHO projects Thailand becomes super-aged — 28% over 60 — by **2035** [WHO 2024]. By 2050 roughly 36% of the population will be 60+ [World Bank 2023]. Longevity raises the duration of health and care liabilities insurers must underwrite.

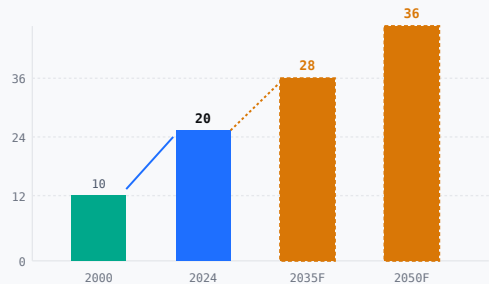
Regional divergence

The North, Central, and North-East regions already register the highest ageing indices [GISTDA 2025], driven by low fertility and out-migration of working-age adults. Provincial micro-markets mature at different speeds, shaping where rural distribution investment pays back.

Share of population aged 60 and over

% 60+

THAILAND · % OF POPULATION · 2000–2050F



SOURCE: WORLD BANK, GISTDA, WHO · KIRA ANALYSIS · KIRA RESEARCH 2026

CHILDREN UNDER 15, 2024

9.5 m
below the 60+ cohort

GISTDA

SUPER-AGED THRESHOLD

2035
28% aged 60+ [WHO 2024]

WHO

60+ SHARE BY 2050

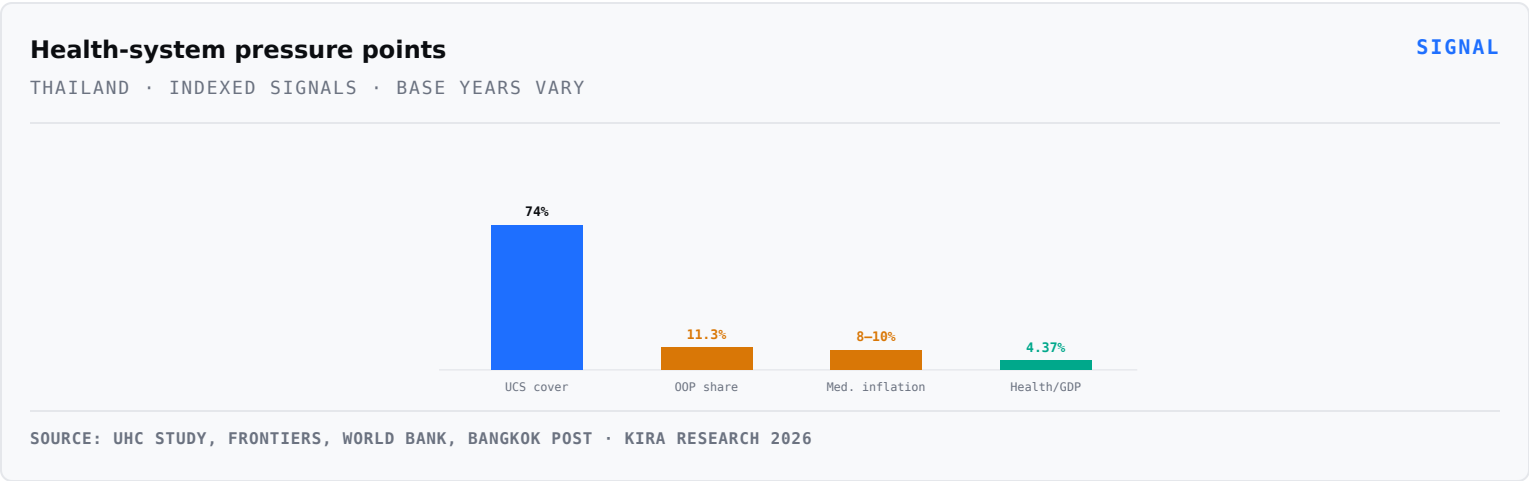
36 %
from 10% in 2000

WORLD BANK

SOURCE KEY · GISTDA 2025 = GISTDA population analysis 2025 · WHO 2024 = WHO South-East Asia healthy-ageing feature 2024 · World Bank 2023 = World Bank, Caring for Thailand's Aging Population 2023

Where universal coverage ends and private cover begins

Thailand's tax-financed Universal Coverage Scheme is a genuine achievement — but breadth is not the same as depth. As the population ages and medical costs outrun general inflation, the **private health protection gap** becomes the defining commercial opportunity.



Breadth without depth

The UCS covers about **74% of the population** [Frontiers 2024], yet budgets lag the cost of care, and bypassing to private providers pushes out-of-pocket spending to **11.3% of health outlays** [UHC Study 2024].

UCS population covered	~74%
Out-of-pocket share	11.3%
Health spend / GDP 2023	4.37%
Medical inflation	8-10%

WHY THE GAP WIDENS

Health expenditure sat at **4.37% of GDP in 2023**, rising only to ~4.49% by 2028 [World Bank 2023], against a Ministry framework capping total health spend at 5% of GDP. Yet long-term-care costs for dependent elderly are projected to reach **THB 500 bn by 2040**, with elderly healthcare costs set to triple by 2032 [NER 2024]. The fiscal envelope cannot absorb the ageing curve alone.

THE COMMERCIAL READ

The middle class increasingly self-funds the difference between basic public care and the standard it expects — private rooms, faster access, chronic-disease management. That is exactly the demand health riders and standalone health policies capture, explaining the **13.7% premium growth** in the segment [TLAA 2024]. The protection gap is not a market failure to lament; it is the addressable pool to underwrite responsibly.

SOURCE KEY • Frontiers 2024 = Frontiers in Public Health, low use of UCS 2024 • NER 2024 = Development Economic Review, future health expenditure & long-term care 2024 • TLAA 2024 = Thai Life Assurance Association statistics 2024 • UHC Study 2024 = Universal Coverage Scheme out-of-pocket study 2024 • World Bank 2023 = World Bank Thailand ageing & expenditure 2023

Premiums by line: health riders and annuities lead the mix

Life dominates the premium pool, but the growth is concentrated in protection and retirement products. **Whole-life accumulation is mature**; health riders, critical illness, and annuities carry the demographic tailwind into 2027.

Life leads, non-life steadies

Life premium of **THB 654 bn (+3.2%)** dwarfs non-life at THB 286 bn (+0.5%) [TLAA 2024] [TGIA 2024]. Within non-life, health rose a striking 24% in H1 2025 while motor crept up 2% [TGIA 2025] — the ageing signal is visible across both books.

Riders and annuities are the engine

Whole-life accounts for ~60% of life GWP, but supplementary riders hold **22.3% and grow faster** [GlobalData 2025]. Health and CI riders are forecast at a 5.4% CAGR to 2028 [GlobalData 2025]. Annuities — paying to ages 90-99 — are the natural retirement-income answer for a super-aged society [Muang Thai 2025].

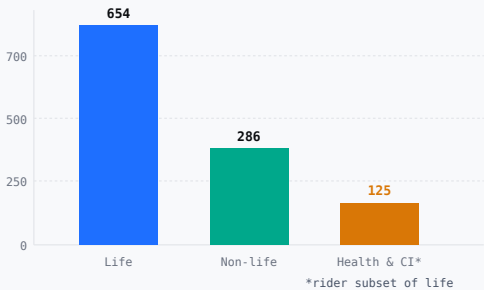
The USD frame

The total market is about **USD 31 bn in 2024**, projected to USD 38 bn by 2029 at a 4.1% CAGR [Statista 2025]; the life sector alone is forecast to reach USD 23 bn by 2028 [GlobalData 2025].

2024 premium pool by line

THB BN

THAILAND · THB BN · LIFE VS NON-LIFE VS HEALTH-CI



SOURCE: TLAA, TGIA, GLOBALDATA, STATISTA · KIRA ANALYSIS · KIRA RESEARCH 2026

LIFE PREMIUM 2024

654 THB bn

+3.2% YoY [TLAA 2024]

TLAA

NON-LIFE HEALTH, H1 2025

+24%

fastest non-life line [TGIA 2025]

TGIA

MARKET BY 2029

38 USD bn

4.1% CAGR [Statista 2025]

STATISTA

SOURCE KEY · GlobalData 2025 = GlobalData Thailand life insurance market analysis 2025 · Muang Thai 2025 = Muang Thai Life annuity product disclosures 2025 · Statista 2025 = Statista Thailand insurance market forecast 2025 · TGIA 2024 = Thai General Insurance Association statistics 2024 · TGIA 2025 = Thai General Insurance Association H1 2025 update · TLAA 2024 = Thai Life Assurance Association statistics 2024

SECTION 10

Competitive structure

Thai life insurance is a scale game. The top three carriers — **AIA, FWD, and Thai Life** — write more than half of all premium, and the demographic shift rewards balance-sheet depth, distribution trust, and product breadth across the ageing lifecycle.

10.1 MARKET SHARES

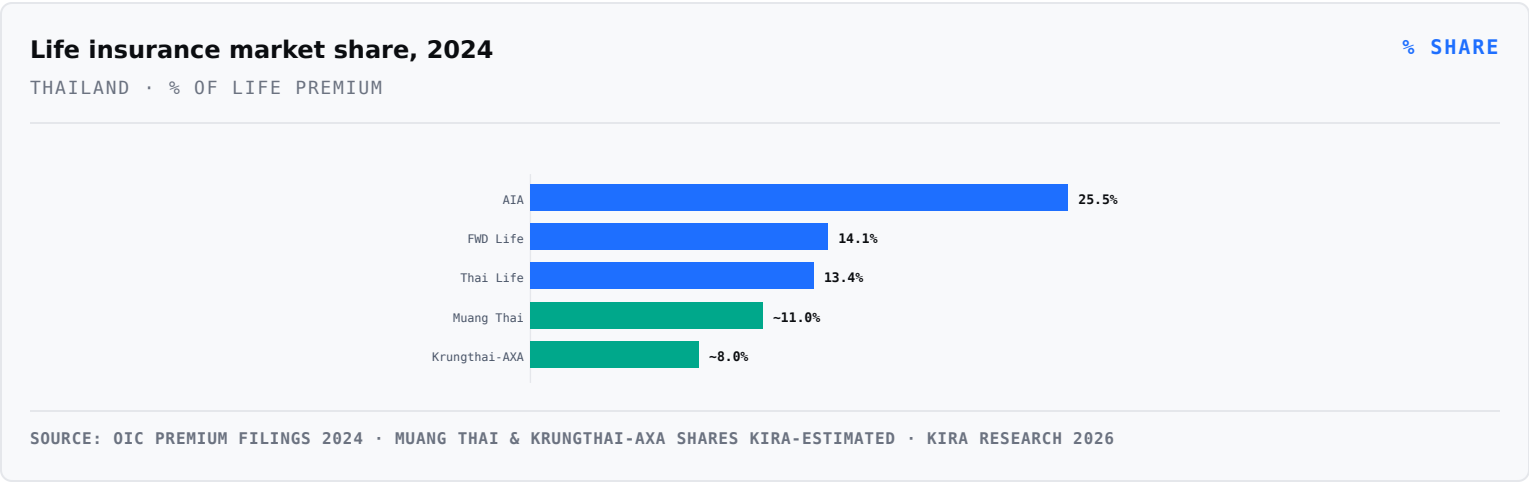
10.2 TOP 5 PLAYERS

10.3 LEADER PROFILE

10.4 DISTRIBUTION

Market structure: concentrated, foreign-anchored, scaling

The top three life insurers control roughly **53% of premium** [OIC 2024], a moderately-to-highly concentrated structure. International parents bring capital and actuarial depth; the demographic transition widens the gap between scale incumbents and the long tail.



Scale and parentage

AIA leads with **25.5% (THB 167 bn)**, FWD 14.1% (THB 92 bn), Thai Life 13.4% (THB 88 bn) [OIC 2024]. Foreign and bank-linked parents underpin the leaders' capital strength.

Top 3 combined share	~53%
AIA premium	THB 167 bn
FWD premium	THB 92 bn
Muang Thai premium	THB 72 bn

MARKET LEADER

RANK 01

AIA Thailand

AIA Group (Hong Kong)

Share

25.5%

Premium

THB 167 bn

Channel

Agency-led

Strengths:

Largest agent force, health & protection focus, regional balance-sheet depth.

RANK 02

FWD Life Insurance

FWD Group (Pan-Asian)

Share

14.1%

Premium

THB 92 bn

Channel

Banca + digital

Strengths:

Digital-first brand, strong bancassurance, fast-growing protection book.

RANK 03

Thai Life Insurance

Local listed (SET: TLI)

Share

13.4%

Premium

THB 88 bn

Channel

Agency-led

Strengths:

Deep domestic brand, senior-focused products, listed-company transparency.

RANK 04

Muang Thai Life

Muang Thai Group / Ageas tie

Premium

THB 72 bn

Focus

Annuity, retirement

Channel

Banca (Kasikorn)

Strengths:

Retirement-product depth (annuity to 99), strong bank distribution.

AIA Thailand

Subsidiary of AIA Group · Hong Kong-listed pan-Asian insurer

MARKET LEADER

HEALTH & PROTECTION

KIRA TIER 1

LIFE SHARE

25.5 %

PREMIUM

167 THB bn

PRIMARY CHANNEL

Agency

PARENT

AIA Grp

MARKET POSITION

AIA is the clear leader of Thai life insurance, writing **THB 167 bn and 25.5% of premium in 2024** [OIC 2024] — nearly double the next carrier. Its scale is built on the country's largest tied-agent force and a long-running emphasis on health and protection rather than pure savings products.

WHY IT WINS THE AGEING CUSTOMER

The leader's product depth across health riders, critical illness, and senior life maps directly onto the demographic shift. As health and CI premiums grow **13.7% a year** industry-wide [TLAA 2024], AIA's protection tilt positions it to capture a disproportionate share of that pool.

STRATEGIC POSTURE

Group capital strength supports long-duration health and annuity guarantees under the tightening RBC and IFRS 17 regime [OIC 2025]. AIA pairs the trust of face-to-face agents — the channel older buyers prefer — with steady investment in digital underwriting and claims, positioning it to defend share as the market re-platforms.

STRENGTHS & WATCHLIST

- **Agent scale:** largest face-to-face force, decisive for senior buyers
- **Protection tilt:** health & CI focus rides the demographic tailwind
- **Capital depth:** regional parent eases RBC / IFRS 17 burden
- **Watch:** FWD's digital banca momentum; copayment communication risk

SOURCE KEY · OIC 2024 = Office of Insurance Commission life-premium filings 2024 · OIC 2025 = Office of Insurance Commission 2025 directives (RBC, IFRS 17) · TLAA 2024 = Thai Life Assurance Association statistics 2024

SECTION 11

Regulation & the **cost-control** **turn**

The Office of Insurance Commission has pivoted toward sustainability. The **copayment clause of March 2025**, alongside risk-based capital and IFRS 17, signals a regime that prizes loss-ratio discipline and balance-sheet strength as medical costs climb.

11.1 COPAYMENT

11.2 RBC & IFRS 17

11.3 NEW HEALTH STANDARD

11.4 WINNERS & LOSERS

A regime tuned for sustainability, not just growth

The OIC's 2025 agenda — copayment, risk-based capital, IFRS 17, and ESG — is explicitly about **long-term solvency** as the demographic and medical-cost curves steepen. Carriers that align pricing, capital, and disclosure early will out-earn those that resist.

COPAYMENT: THE DEMAND-SIDE BRAKE

Effective for individual policies issued on or after **19 March 2025** [OIC 2025], the copayment clause shares claims cost with policyholders to control over-utilisation and stabilise premium rates. Built under the New Health Standard (OIC Orders 14–15/2564), it is a deliberate tool against medical inflation of **8–10% a year** [Bangkok Post 2024].

RBC AND IFRS 17

Risk-based capital and IFRS 17 sharpen the cost of long-duration guarantees [OIC 2025]. Annuity and whole-life books now carry transparent capital charges, advantaging insurers with disciplined asset-liability management and deep parental capital.

WHAT IT MEANS FOR PARTICIPANTS

The cost-control turn favours **scale, actuarial sophistication, and clear customer communication**. Carriers that explain copayment well retain trust and protect loss ratios; those that surprise policyholders risk lapse and anti-selection.

- **Winners:** capital-strong, health-focused leaders with strong actuarial functions
- **Pressured:** sub-scale insurers with thin capital under RBC / IFRS 17
- **Pivotal:** distribution that can explain copayment without eroding trust
- **Emerging:** standardised products easing comparison and price competition

The OIC's broader 2025 framework lists 13 mechanisms to strengthen the financial system against new global risks [OIC 2025] — a clear signal that resilience, not headline growth, defines the next cycle.

SOURCE KEY · OIC 2025 = Office of Insurance Commission 2025 industry directives (copayment, RBC, IFRS 17, New Health Standard) · Bangkok Post 2024 = Bangkok Post insurer medical-inflation report 2024

Technology shifts from pilot to underwriting core

AI in underwriting, claims, and fraud detection has moved from differentiator to **table stakes** across the global industry [InsurTech 2025]. For Thai insurers serving an ageing book, the prize is faster onboarding, controlled loss ratios, and channel economics that keep trusted human advisers viable.

From experiment to infrastructure

Leading insurers and insurtechs now run machine-learning models across customer service, underwriting, claims, and fraud prediction [InsurTech 2025]. By 2026 these are no longer competitive advantages but baseline capability, and Thai carriers are integrating them into core workflows.

Fraud and loss-ratio control

AI fraud-detection has cut fraudulent claims by as much as **~30% in documented deployments** [InsurTech 2025]. Paired with the copayment regime, this gives Thai health insurers a second lever against medical-cost escalation.

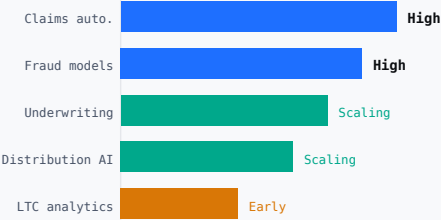
Channel: augment, don't replace

Older buyers still trust face-to-face advice and bancassurance. The realistic Thai pattern is **AI-augmented agents and banca** — digital tools speeding quotes, underwriting, and claims while the human relationship anchors the sale.

Where AI lands first in Thai insurance

MATURITY

ADOPTION MATURITY · DIRECTIONAL READ



SOURCE: INSURTECH DIGITAL, NORTON ROSE FULBRIGHT · KIRA ANALYSIS · KIRA RESEARCH 2026

FRAUD-CLAIM REDUCTION

~30%
leading AI deployments [InsurTech 2025]

INSURTECH

STATUS BY 2026

Table stakes
AI claims & underwriting [InsurTech 2025]

INSURTECH

THAI CHANNEL PRIORITY

Banca + agent
trust-led for older buyers

KIRA EST.

SOURCE KEY · InsurTech 2025 = InsurTech Digital, AI claims & underwriting review 2025 · Kira estimates = KIRA in-house analyst triangulation

Six technology applications reshaping Thai insurers

Translation into operating practice will separate 2027 winners from laggards. These use cases span **customer acquisition** through **claims control**, each observable in Thailand or its closest regional benchmarks today.

01 • UNDERWRITING

Algorithmic health underwriting

ML models price health and CI risk for older applicants faster and more granularly, expanding insurable ages that historically faced blanket exclusions. Critical for capturing the 60+ pool profitably.

BENCHMARK: PAN-ASIAN CARRIERS

02 • CLAIMS

Straight-through claims automation

AI triages and settles routine health claims without manual touch, cutting cycle time and cost. Paired with copayment rules, it enforces consistent, auditable adjudication at scale.

STATUS: HIGH MATURITY GLOBALLY

03 • FRAUD

Claims fraud detection

Pattern models flag anomalous health claims, reducing fraudulent payouts by up to **~30%** in leading deployments [InsurTech 2025]. A direct loss-ratio lever as medical costs rise.

ANCHOR: ~30% FRAUD REDUCTION

04 • DISTRIBUTION

Agent & banca enablement

Recommendation engines surface the right annuity or health rider for each customer at the point of advice, lifting cross-sell across the ageing lifecycle while keeping the trusted human in the loop.

OBSERVABLE: BANCASSURANCE LEADERS

05 • RETENTION

Lapse & persistency modelling

Predictive models identify policies at risk of lapse — heightened where copayment changes expectations — enabling proactive outreach that protects long-duration health and annuity books.

TIE-IN: COPAYMENT COMMUNICATION

06 • CARE ANALYTICS

Long-term-care risk analytics

Emerging models project dependency and care-cost trajectories for the elderly — foundational for pricing LTC products against the **THB 500 bn** 2040 care-cost wave [NER 2024].

STAGE: EARLY, HIGH STRATEGIC VALUE

SOURCE KEY • InsurTech 2025 = InsurTech Digital, AI claims & underwriting review 2025 • NER 2024 = Development Economic Review, long-term care cost projection 2024

Outlook to 2032: modest headline, decisive mix shift

We expect Thai premium growth to stay in the **low-to-mid single digits**, with the demographic transition driving above-trend expansion in health riders, annuities, and — late in the horizon — long-term care. The carriers that win are those that reposition toward the ageing customer now.

Base case

Industry premium compounds around **4% to 2029** [Statista 2025], with life reaching ~USD 23 bn by 2028 [GlobalData 2025]. Health and CI outpace the market at high single to low double digits [TLAA 2024] [GlobalData 2025] as the protection gap and medical inflation persist.

Upside & downside

Upside: faster middle-class uptake of private health, tax-incentivised annuities, and LTC product innovation. Downside: an economic slowdown compressing discretionary protection spend, or copayment friction depressing health-rider sales near term.

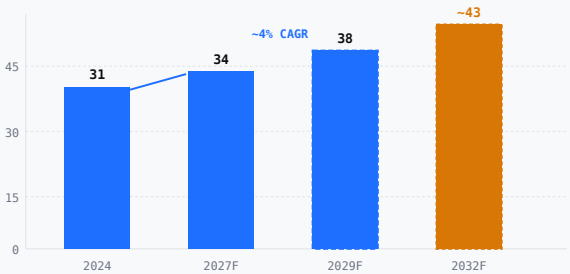
The strategic imperative

The demographic curve is the most reliable variable in this market. **Reposition the book toward the 60+ lifecycle** — health, annuity, LTC — and invest in capital strength and AI-augmented distribution. Sub-scale, savings-tilted insurers face the steepest adjustment.

Total premium trajectory

USD BN

THAILAND · USD BN · 2024–2032F



SOURCE: STATISTA, GLOBALDATA · 2032 FIGURE KIRA-ESTIMATED FROM ~4% CAGR · KIRA RESEARCH 2026

MARKET 2027 (BASE)

~34 USD bn
on ~4% CAGR [Kira estimates]

KIRA EST.

LIFE SECTOR 2028

23 USD bn
[GlobalData 2025]

GLOBALDATA

LTC COST WAVE 2040

500 THB bn
dependent-elderly care [NER 2024]

NER

SOURCE KEY · GlobalData 2025 = GlobalData Thailand life insurance market analysis 2025 · NER 2024 = Development Economic Review long-term care projection 2024 · Statista 2025 = Statista Thailand insurance forecast 2025 · TLAA 2024 = Thai Life Assurance Association statistics 2024 · Kira estimates = KIRA in-house analyst triangulation

Methodology & source index

This analysis blends regulatory and association statistics, demographic and health-system research, and operator disclosures with KIRA analyst triangulation. Forward figures are directional scenarios anchored to documented base years, not point predictions. Roughly a fifth of quantitative claims are KIRA estimates; the remainder cite named external sources.

SOURCE INDEX

[1] Thai Life Assurance Association — life premium statistics and annual report 2024.

[2] Thai General Insurance Association — non-life direct-premium statistics 2024 and H1 2025.

[3] Office of Insurance Commission — 2024 premium filings; 2025 industry directives, copayment clause, RBC and IFRS 17.

[4] GISTDA — 2025 population analysis on elderly versus child cohorts.

[5] World Health Organization — South-East Asia healthy-ageing feature, super-aged projection to 2035.

[6] World Bank — Caring for Thailand's Aging Population; health expenditure series 2023.

[7] Universal Coverage Scheme utilisation and out-of-pocket study, 2024.

[8] Frontiers in Public Health — determinants of low UCS use, 2024.

[9] Development Economic Review — future health expenditure and long-term care costs, 2024.

[10] Statista — Thailand insurance market forecast, 2025.

[11] GlobalData — Thailand life insurance market analysis and 2028 forecast, 2025.

[12] Bangkok Post — insurer growth and medical-inflation reporting, 2024.

[13] Muang Thai Life — annuity and retirement product disclosures, 2025.

[14] InsurTech Digital — generative AI in claims and underwriting, 2025.

[15] KIRA Research — in-house analyst triangulation across the above.